



OPERATIVE REPORT: LEFT ABOVE-ELBOW AMPUTATION

PATIENT	DOB	MRN	DATE OF SERVICE
Jonathan Davis	10/14/1995	948-20491-A	01/08/2024

SURGEON: Dr. Marcus Vance, MD, FACS

ASSISTANT: Dr. David Cho, MD

PREOPERATIVE DIAGNOSIS: Mangled, unsalvageable left upper extremity, traumatic sub-total amputation at elbow.

POSTOPERATIVE DIAGNOSIS: Status post left transhumeral (above-elbow) amputation.

PROCEDURE PERFORMED:

Left transhumeral amputation with primary myofascial closure, bone beveling, traction-neurectomy of the median, ulnar, and radial nerves, and primary closure.

OPERATIVE NARRATIVE:

The patient was taken emergently to Operating Room 3. General anesthesia was induced. The left shoulder and stump area were prepped and draped in the standard sterile fashion. The limb was inspected; there was severe contamination from machine grease and metallic dust.

Fish-mouth incisions were planned in the healthy skin overlying the middle third of the humerus to secure adequate coverage. Skin flaps were developed. Muscles of the anterior and posterior compartments were identified and sectioned. The brachial artery was identified, doubly ligated with 2-0 silk sutures, and divided.

The median, ulnar, and radial nerves were isolated. Each nerve was dissected proximally under tension, injected with 0.5% bupivacaine, transected under high traction, and allowed to retract deeply into the muscular bed to mitigate future neuroma formation.

The humerus was sectioned at the junction of the middle and distal thirds using an oscillating saw under continuous saline irrigation. Sharp edges were rasped smooth. Copious irrigation with 9 liters of normal saline containing bacitracin was performed to decontaminate the wound.

Myofascial closure was accomplished by securing the triceps fascia to the anterior biceps fascia over the bony stump. Redundant skin flaps were trimmed. A JP drain was placed deep to the myofascial layer. Skin was closed with skin staples. Dressing applied.

ESTIMATED BLOOD LOSS: 650 mL.

FLUID REPLACEMENT: 2 units Packed Red Blood Cells, 1 Liter Lactated Ringers.

DISPOSITION: Recovery room, then to ICU for hemodynamic monitoring.

NURSING FLOWSHEET — VITAL SIGNS

ICU / TRAUMA WARD · SHEET 1 OF 1

PATIENT		DOB		MRN		DATE OF SERVICE	
Jonathan Davis		10/14/1995		948-20491-A		01/09/2024	

DATE/TIME	BP		HR	RR	SPO2	TEMP °F	PAIN	POSITION	NEURO ✓	INIT
01/09 00:00	154/88		98	18	95%	99.0	10	HOB 30°	WNL	JM
01/09 08:00	138/91		92	16	97%	98.7	7	chair	WNL	RB
01/09 16:00	139/88		90	15	97%	98.5	7	amb w/ PT	WNL	TK
01/10 00:00	131/87		82	16	98%	98.5	5	supine	WNL	JM

Pain reassessed within 60 minutes of PRN analgesia per policy. Neuro checks q4h while concussion precautions active.

MEDICATION ADMINISTRATION RECORD (MAR) SHEET 1 OF 4

PATIENT Jonathan Davis	DOB 10/14/1995	MRN 948-20491-A	DATE OF SERVICE 01/08/2024
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MEDICATION / ORDER	01/08/2024						
	02:00	06:00	08:00	10:00	14:00	18:00	22:00
Hydromorphone (Dilaudid) PCA	—	—	JM	—	—	—	—
Gabapentin 300mg PO	—	—	JM	—	—	—	—
Ancef (Cefazolin) 1g IV	—	PV	—	—	TK	—	CO
Oxycodone 5-10mg PO [PRN]	—	—	—	—	—	—	—

MEDICATION / ORDER	01/09/2024						
	02:00	06:00	08:00	10:00	14:00	18:00	22:00
Hydromorphone (Dilaudid) PCA	—	—	JM	—	—	—	—
Gabapentin 300mg PO	—	—	JM	—	—	—	—
Ancef (Cefazolin) 1g IV	—	PV	—	—	TK	—	CO
Oxycodone 5-10mg PO [PRN]	—	—	—	—	—	—	—

MEDICATION ADMINISTRATION RECORD (MAR) SHEET 2 OF 4

PATIENT Jonathan Davis	DOB 10/14/1995	MRN 948-20491-A	DATE OF SERVICE 01/08/2024
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MEDICATION / ORDER	01/10/2024						
	02:00	06:00	08:00	10:00	14:00	18:00	22:00
Hydromorphone (Dilaudid) PCA	—	—	TK	—	—	—	—
Gabapentin 300mg PO	—	—	TK	—	—	—	—
Ancef (Cefazolin) 1g IV	—	RB	—	—	PV	—	CO
Oxycodone 5-10mg PO [PRN]	—	—	—	—	—	—	—

MEDICATION / ORDER	01/11/2024						
	02:00	06:00	08:00	10:00	14:00	18:00	22:00
Hydromorphone (Dilaudid) PCA	—	—	TK	—	—	—	—
Gabapentin 300mg PO	—	—	TK	—	—	—	—
Ancef (Cefazolin) 1g IV	—	RB	—	—	PV	—	CO
Oxycodone 5-10mg PO [PRN]	—	—	—	—	—	—	—

MEDICATION ADMINISTRATION RECORD (MAR) SHEET 3 OF 4

PATIENT Jonathan Davis	DOB 10/14/1995	MRN 948-20491-A	DATE OF SERVICE 01/08/2024
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MEDICATION / ORDER	01/12/2024						
	02:00	06:00	08:00	10:00	14:00	18:00	22:00
Hydromorphone (Dilaudid) PCA	—	—	TK	—	—	—	—
Gabapentin 300mg PO	—	—	TK	—	—	—	—
Ancef (Cefazolin) 1g IV	—	CO	—	—	PV	—	PV
Oxycodone 5-10mg PO [PRN]	—	—	—	—	—	—	—

MEDICATION / ORDER	01/13/2024						
	02:00	06:00	08:00	10:00	14:00	18:00	22:00
Hydromorphone (Dilaudid) PCA	—	—	TK	—	—	—	—
Gabapentin 300mg PO	—	—	TK	—	—	—	—
Ancef (Cefazolin) 1g IV	—	CO	—	—	PV	—	PV
Oxycodone 5-10mg PO [PRN]	—	—	—	—	—	—	—

MEDICATION ADMINISTRATION RECORD (MAR) SHEET 4 OF 4

PATIENT Jonathan Davis	DOB 10/14/1995	MRN 948-20491-A	DATE OF SERVICE 01/08/2024
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MEDICATION / ORDER	01/14/2024						
	02:00	06:00	08:00	10:00	14:00	18:00	22:00
Hydromorphone (Dilaudid) PCA	—	—	RB	—	—	—	—
Gabapentin 300mg PO	—	—	RB	—	—	—	—
Ancef (Cefazolin) 1g IV	—	JM	—	—	TK	—	TK
Oxycodone 5-10mg PO [PRN]	—	—	—	—	—	—	—

Initials: PV = P. Villanueva RN · CO = C. Okafor RN · JM = J. Marsh RN · RB = R. Bell RN · TK = T. Kovac RN · H = held (criteria not met) · — = not scheduled

LABORATORY REPORT — FINAL

PATIENT	DOB	MRN	DATE OF SERVICE
Jonathan Davis	10/14/1995	948-20491-A	01/08/2024

ACCESSION	LAB-240108-92	SPECIMEN	Blood
COLLECTED	01/08/2024 2024-01-08 13:40	ORDERING PROVIDER	—
RECEIVED	01/08/2024 2024-01-08 13:50	REPORTED	01/08/2024 2024-01-08 14:15

COMPLETE BLOOD COUNT & TOXICOLOGY			
TEST	RESULT	UNITS	REFERENCE RANGE
WBC	14.2 (H)	K/uL	4.0-11.0
Hgb	8.9 (L)	g/dL	13.5-17.5
Hct	27.2 (L)	%	41.0-50.0
PLT	210	K/uL	150-450
Urine Drug Screen: Cannabinoids	Positive (Trace) (A)	ng/mL	Negative
Urine Drug Screen: Ethanol	Negative	mg/dL	Negative

H = above reference · L = below reference · A = abnormal — reviewed and released by laboratory staff. Performing lab: Midwest Regional Medical Center Clinical Laboratory, CLIA 36D-0000000 (synthetic).



INPATIENT DISCHARGE SUMMARY

PATIENT	DOB	MRN	DATE OF SERVICE
Jonathan Davis	10/14/1995	948-20491-A	01/15/2024

PATIENT: Jonathan Davis

ADMIT DATE: 01/08/2024

DISCHARGE DATE: 01/15/2024

HOSPITAL COURSE:

The patient was admitted following a emergent left transhumeral amputation for severe industrial crush trauma. Postoperatively, he was monitored in the surgical ICU for 24 hours. Hemoglobin stabilized post-transfusion. He was transitioned from IV PCA Dilaudid to oral Oxycodone and Gabapentin 300mg TID. Wound healing was inspected daily; skin flaps remain viable, dry, and clean. JP drain removed on postoperative day 4. Physical Therapy and PM&R (Dr. Elena Rostova) consulted to coordinate future prosthetic fit and phantom pain management.

DISCHARGE PLAN & INSTRUCTIONS:

1. Wound Care: Keep stump clean and dry. No soaking. 2. Medications: Gabapentin 300mg PO TID for phantom/neuropathic pain. Oxycodone 5mg PO q4-6h prn pain. Cefdinir 300mg PO BID for 5 more days. 3. Follow-up: Dr. Marcus Vance in Orthopedic Clinic in 1 week. Contact Dr. Elena Rostova for prosthetic evaluation in 4 weeks.

